

Surgical Date:

Gross Description: A. "Right axilla palpable node." A 2.2 x 1.5 x 0.6 cm portion of lymph node bearing fat contains a 0.5 x 0.5 x 0.5 cm hard white nodule. Imprints are prepared, and a portion is selected for frozen section examination. B. "Right breast stitch marks axillary tail." A he 14.9 x 13.5 x 4.0 cm right breast with an overlying 8.0 x 4.0 cm keyhole-shaped skin. The areolar complex is unremarkable. There is a 0.3 cm skin lesion (grossly consistent with seborrheic keratosis), 0.5 cm and located 0.6 cm medial to the nipple. There is a stellate mass within the central breast 5.7 x 4.0 x 2.0 cm, located 2.0 cm from the deep margin and occupies the upper and lower outer quadrants as well as the upper inner and outer quadrants. There is a biopsy marker recognize in the lower outer quadrant (8 o'clock aspect), 3.7 cm from the deep margin. A second biopsy marker is recognized and approximately 1 o'clock, 3.5 cm from the deep margin. Superior to the main mass is a satellite nodule attached by fibrous strands, 0.7 x 0.5 x 0.5 cm, located 2.0 cm from the deep margin and 1.8 cm superior to the main mass at approximately 12 o'clock. The remaining breast tissue is centrally dense. There are no intramammary lymph nodes. The margin is a blue. C. "Level 1 and 2 right axillary lymph nodes." A 7.0 x 5.0 x 2.0 cm aggregate of adipose tissue fragments containing 11 lymph nodes up to 1.5 cm. The lymph nodes all appear involved by tumor with the largest metastasis measuring 1.4 cm. Extracapsular extension is noted.

Microscopic Description: Biopsy type: Mastectomy

Tumor site: Central

Size: 5.7 cm

Specimen Margin: Free of invasive carcinoma by 20 mm (deep)

Tumor grade (SBR/Nottingham): N/A

Tubule formation: N/A

Nuclear atypia: 2

Mitotic rate: 2 (focal)

Lymphovascular invasion: Present (focal)

In situ component: Lobular carcinoma in situ

Calcifications: Absent

Non-neoplastic breast: Atrophic with intramammary metastases

Estrogen (ER) and Progesterone (PR) Receptors (IMPX): ER strongly positive, PR strongly positive on previous biopsy
Her-2/Neu (H2N)

ICD-O. 3
Carcinoma, infiltrating
lobular NOS 8520/3
Site (R) Breast NOS
C50.9
JTD 6/10/13

UUID: 649E6215-36F9-4BEB-9EE9-683E4FC75C42
TCGA-E9-A6HE-01A-PR Redacted



Amplification: Negative on previous biopsy

Nipple involvement: Present

Skin involvement: No

Skeletal muscle: Not present

Lymph nodes: Axillary (specimens A and C): 12 metastases (12 nodes examined) The largest metastasis is 14 mm; there is extensive extracapsular extension.

PTNM Stage: pT3 N3

Diagnosis Details: Subcutis (right axilla): Metastatic invasive lobular carcinoma (presumable an effaced lymph node) Breast (right): Invasive lobular carcinoma, pleomorphic type with intramammary and axillary metastases

Comments: Multiple axillary lymph nodes exhibit effacement by invasive lobular carcinoma, with extensive involvement of extranodal adipose tissue. Invasive lobular carcinoma extends to the cauterized margin of axilla specimen

Formatted Path Reports: BREAST TISSUE CHECKLIST

Specimen type: Mastectomy

Specimen size: Not specified

Tumor site: Breast

Tumor size: 5.7 cm

Grossly evident lesion: Yes

Histologic type: Infiltrating lobular carcinoma

Histologic grade: Not specified

Tumor extent: Nipple

Lymph nodes: 12/12 positive for metastasis (Axillary 12/12)

Extracapsular invasion of the lymph nodes: Not specified

Margins: Uninvolved

Nottingham Histologic Score

Tubule formation: Not specified

Nuclear pleomorphism: Not specified

Mitotic count (25x): Not specified

Mitotic count (40x): Not specified

Total Nottingham Score: Score cannot be determined

Evidence of neo-adjuvant treatment: Not specified

Additional pathologic findings: Not specified

Comments: Right breast

Criteria	Yes	No
Immunohistochemistry		✓
Primary Tumor Site Discrepancy		✓
IPSSA Discrepancy		✓
Prior Pregnancy history		✓
Discrepancy between Primary and Secondary Site		✓
Case Review	W	
Reviewer Initials	KML	
Date Reviewed	4/3/13	

QUALIFIED / DISQUALIFIED